Because Cefuroxime is excreted in human milk, consideration should be given to discontinuing nursing temporarily during treatment with cefuroxime. Using **SITEX** Tablet in absence of a proven or strongly suspected bacterial infections or a prophylactic indications is unlikely to provide benefits to the patients and increases the risk of the development of drug-resistant bacteria Cephalosporins including Cefuroxime should be given with caution to patients receiving concurrent treatment with potent diuretics because these diuretics are suspected of a adversely affecting renal function.

Cefuroxime as with other broad-spectrum antibiotics should be prescribed with caution in individual with a history of Colitis.

DRUG INTERACTIOS:
Antacid: Drug that reduce gastric acidity may result in lower bioavailability of **SITEX** compared with that of fasting state and tend to cancel the effect of postprandial absorption.

Oral contraceptives: In common with other antibiotics, Cefuroxime Axetil may affect the gut flora, leading to lower oestrogen reabsorption and reduced efficacy of combined oral oestrogen / progesterone.

STORAGE:
Store below 25°C. Protect from light and moisture in a dry place.
Keep medicine out of reach of children.

PRESENTATION:
A Blister of 10 Tablets.

NAFDAC Reg. No.:

Manufactured by:
Coral Laboratories Ltd.
Plot No. 27-28, Pharmacity, Selaqui,
Dehradun - 248 011, Uttarakhand, INDIA.
Email: exports@corallab.com
Website : www.corallab.com

Manufactured for:
SEAL HEALTHCARE LIMITED
2, Ashimolowo Street, Off 5th Avenue,
Abesan Estate, Ipaja, Lagos State, Nigeria.

SITEX TABLETS

Cefuroxime Axetil and Potassium Clavulanate Tablets

COMPOSITION:
Each film coated tablet contains:
Cefuroxime Axetil BP
Equivalent to Cefuroxime ..... 500 mg
Diluted Potassium Clavulanate BP
Equivalent to Clavulanic Acid ..... 125 mg
Excipients ..... Q.S.
Colour: Sunset Yellow FCF & Titanium Dioxide BP

MECHANISM OF ACTION:
Cefuroxime has bactericidal activity against a wide range of common pathogens, including beta-lactamase producing strains. The bactericidal action of cefuroxime results from Inhibition of cell wall synthesis by binding to essential target proteins. Cefuroxime has good stability to bacterial beta lactamases.

Clavulanic Acid is a naturally derived bola lactamase Inhibitor produced by Streptomyces clavuligerus. Clavulanic Acid binds to and inactivates them thus, preventing the destruction of cefuroxime that is a substrate for this enzyme. It has poor intrinsic antimicrobial activity, but It Is Irreversible binder of Beta lactamases produced by a Wide range of gram positive and gram-negative microorganism.

Rationale for Combination:
Clavulanic Acid's principal role is as a beta lactamase Inhibitor. Beta-lactam antibiotics, such as the penicillin and cephalosporins, act by disrupting the development of bacterial cells walls thus, causing the disintegration of the bacteria, However, some bacteria acquire the genes to produce enzymes which Inactivate this mode of action so called beta-lactamases - drastically reducing the efficacy of this class or antibiotics.

Clavulanic Acid has a similar structure to the beta-lactam antibiotics but binds irreversibly to the beta-lactamase enzymes. Used in combination with the beta-lactam antibiotics, It has become one of the most prescribed antibiotics prolonging the effective life of antibiotics, Thus, the combination of Cefuroxime and Clavulanic Acid (Beta lactamase Inhibitor) provide a solution for treatment of bacterial Infections caused by beta lactam resistant pathogens.

INDICATION:
SITEX Tablet are Indicated for the treatment of patients with mild to moderate infections caused by susceptible strains of the designated microorganisms in the conditions listed below:

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SIDE 1

Pharyngitis/Tonsillitis caused by Streptococcus pyogenes.

Acute Bacterial Otitis Media caused by Streptococcus pneumonia, Haemophilus influenzae (Including bota-lactamase- producing strains), Moraxella catarrhalis (Including bole -lactamase- producing strains), or Streptococcus pyogenes.

Acute Bacterial Maxillary Sinusitis caused by Streptococcus pneumoniae or Haemophilus Influenzae (non-beta-lactamase-producing strains only).

Acute Bacterial Exacerbations of Chronic Bronchitis and Secondary

Bacterial Infections of acute Bronchitis caused by Streptococcus pneumoniae, Haemophilus influenzae (beta-lactamase negative strains), or Haemophilus para influenzae (beta-lactamase negative strains).

Uncomplicated Skin and Skin-Structure Infections caused by Staphylococcus aureus (including beta-lactamase-producing strains) or Streptococcus pyogenes.

Uncomplicated Urinary Tract Infections caused by Escherichia coli or Klebsiella pneumoniae.

Uncomplicated Gonorrhoea, (urethral and endocervical) caused by penicillinase-producing and non-penicillinase-producing strains of Neisseria gonorrhoeae and uncomplicated gonorrhoea, rectal, in females, caused by non-penicillinase-producing strains of Neisseria gonorrhoeae.

Early Lyme disease (erythema migrans) caused by Borrelia burgdorferi.

DOSAGE:

SITEX Tablets: The usual course of therapy is 5 to 7 days for treatment of bronchitis, and 7 to 10 days for other infections

TABLE 1:

Indications & dosage for SITEX tablets (May be administered without regard to meals.)

Indication	Dosage
Pharyngitis or Tonsillitis	250 mg twice daily 5-10 days
Acute bacterial maxillary sinusitis	250 mg twice daily 10 days
Acute bacterial exacerbation of chronic bronchitis	250-500 mg twice daily 10 days
Secondary bacterial infections of acute bronchitis	250-500 mg twice daily 5-10 days
Community acquired pneumonia	250-500 mg twice daily 5-10 days
Uncomplicated skin & skin-structure infections	250-500 mg twice daily 10 days
MDR Typhoid fever	500 mg twice daily 10-14 days
Uncomplicated urinary tract infection	250 mg twice daily 7-10 days
Uncomplicated gonorrhea	1000 mg single dose
Lyme disease:	500 mg twice daily 20 days
Paediatric patients (3 months to 12 years)	
Pharyngitis or Tonsillitis	20 mg/kg/day in two divided doses 5-10 days
Acute otitis media	30 mg/kg/day in two divided doses 10 days
Acute bacterial maxillary sinusitis	30 mg/kg/day in two divided doses 10 days

PHARMACOKINETICS:

After oral administration cefuroxime axetil is absorbed from the gastrointestinal tract and rapidly hydrolysed in the intestinal mucosa and blood to release cefuroxime into the circulation. Optimum absorption occurs when it is administered shortly after a meal.

Following administration of Cefuroxime Axetil and Potassium Clavulanate Tablets peak serum levels (2.9 µg/ml for a 125 mg dose, 4.4 µg/ml for a 250 mg dose, 7.7 µg/ml for a 500 mg dose and 13.6 µg/ml for a 1000 mg dose) occur approximately 2.4 hours after dosing when taken with food. The pharmacokinetics of cefuroxime is linear over the oral dosage range of 125 to 1000 mg. No accumulation of cefuroxime occurred following repeat oral doses of 250 to 500 mg.

Adverse Effects:

Adverse reactions to cefuroxime Axetil have been generally mild and transient in nature. The following adverse reactions to Cefuroxime Axetil have been reported. However, the possibility of the occurrence of other adverse reactions seen with the Cephalosporins class of antibiotics should be borne ion mind.

Major adverse reactions which may occur are diarrhoea, nausea, transient elevation in AST, ALT, LDH, Eosinophilia. Other adverse events that may occur are abdominal pain, abdominal cramps, flatulence, indigestion, headache, vaginitis, Vulvar itch, rash, hives, itch, dysuria, chills, chest pain, shortness of breath, mouth ulcer, swollen tongue, sleepiness.

WARNINGS/PRECAUIONS:

Before therapy with **SITEX** is instituted. careful inquiry should be made to determine whether the patient has had previous hypersensitivity reactions to cephalosporins, penicillin or other drugs.

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12-06-2026